

رقم البطاقة : 28007250104529 رقم التليفون : 01050523144 تاريخ تسجيل الإستمارة : 30-09-2026
العمر(تاريخ الميلاد) : 25-07-1980 المهنة : ربه منزل

أقر انا الموقع أدناه انني لا اتمتع بأي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-
الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة
نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي (-----)

المقر بما فيه التوقيع: (-----)

[11129]

GRI-E

جوستاف روس

تتعمد صحة الأيايات المرفقة
رئيس اللجنة و مدير
المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

----- /د ----- /د ----- /د.ا ----- /د.ا

التشخيص النهائي :
 خطة العلاج :
 المستشفى :
 قسم :
 نفقات :
 ملاحظات :

رئيس
المجلس الطبي المتخصص

--

Name: اسماء سيد محمود يوسف National ID: 28007250104529	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حميد بالثدى و تحتاج الي CT scan of the chest, abdomen and pelvis.
--

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

PATIENT NAME: ASMAA SAYED MAHMOUD YOSEF

GENDER: Female

DATE : 08-Sep-2026 11:08 AM

PATIENT ID: 10125268924

AGE: 46 y

ORDER: 3997067,3997075

FULL FIELD DIGITAL MAMMOGRAPHY AND COMPLEMENTARY ULTRASOUND

INDICATION:

- Screening.

TECHNIQUE:

- CC and MLO view.
- Complementary ultrasound

REPORT:

RIGHT BREAST:

- Heterogeneously dense breast may obscure small masses (ACR C).
- Right UOQ and axillary scarring with residual operative faint calcifications and no evidence of mass lesions.
- Normal skin thickness and contour.
- Normal nipple areola complexes.
- Right axillary non specific lymph nodes.

LEFT BREAST:

- Heterogeneously dense breast may obscure small masses (ACR C).
- No grouped microcalcifications.
- This was confirmed by Complementary Ultrasound which showed heterogenous echogenic breast parenchyma with no solid or cystic mass lesions.
- Normal skin thickness and contour.
- Normal nipple areola complexes.
- Nonspecific axillary lymph nodes.

OPINION:

- Right breast UOQ post operative changes with related calcifications for correlation with operative data. No underlying suspicious masses.

PATIENT: ASMAA SAYED MAHMOUD YOSEF, 10125268924, Female, 46 y

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مركز بهية الهرم
4 شارع علوية أول شارع الهرم بجوار داري , الجيزة

مستشفى بهية الشيخ زايد
٤٠ الحي الأول - الشيخ زايد أمام كلية الهندسة

PATIENT NAME: ASMAA SAYED MAHMOUD YOSEF

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4 شارع علوية أول شارع الهرم بجوار داري , الجيزة

مستشفى بهية الشيخ زايد
٤٠ الحي الأول - الشيخ زايد أمام كلية الهندسة

PATIENT NAME: ASMAA SAYED MAHMOUD YOSEF

GENDER: Female

DATE : 17-Mar-2026 09:59 AM

PATIENT ID: 10125268924

AGE: 45 y

ORDER: 3796909

BONE MINERAL DENSITY

SITE	Region	BMineral Density (BMD)	T- score
Forearm	Distal	0.641	-0.9
Femur	Neck	0.706	-1.3
Spine	Average	0.975	-0.7

World Health Organization (WHO)	Criteria For osteoporosis
Diagnostic Category	T-Score
Normal	T-Score from +1 to -1
Osteopenia	T-Score from - 1 to - 2.5
Osteoporsis	T-Score from - -2.5
Established osteoporosis	T-Sore < -2.5 (with fractures)

Comment:

Measured BMD values are indicative of:

- **Osteopenia at the femoral neck region.**

Reported By,

Dr. Yasmine Abdel-Naby, MSc

17-Mar-2026 02:29 PM



PATIENT NAME: ASMAA SAYED MAHMOUD YOSEF

GENDER: Female

DATE : 27-Aug-2025 09:52 AM

PATIENT ID: 10125268924

AGE: 45 y

ORDER: 3529723,3535852

Dynamic Contrast Enhanced MRI Study of the Breast

Clinical summary:

- Right breast pathologically proved IDC.

Pulse Sequences:

- Pre-contrast: axial T1, T2, STIR and sagittal T2 WIs.
- Post contrast: axial VIBE and Subtraction WIs.

Findings:

The breasts are heterogeneously dense, showing bilateral moderate symmetrical background parenchymal enhancement.

RIGHT BREAST:

- An axillary tail irregular mass is noted, eliciting low signal in T1 and T2 WIs and heterogeneous bright signal in STIR. It shows rim contrast enhancement, measuring about 1.9x1.6x1.7cm in maximum dimensions, 1.5cm from lateral skin and 14cm from nipple.
- Multiple scattered enhancing foci are noted with no restricted diffusion and no ultrasound correlate.
- UIQ small faint focal non-mass enhancement is noted with no restricted diffusion. Second look ultrasound revealed no correlate.
- Multiple subtle dermal focal thickening with post-contrast enhancement are noted at lower quadrants. 2nd Look ultrasound revealed corresponding subtle focal dermal thickening with overlying skin redness, likely inflammatory.
- Normal nipple areola complexes.
- Few axillary lymph nodes with increased cortical thickening are noted at level I.

LEFT BREAST:

- On post-contrast series, multiple scattered enhancing foci are noted more pronounced at the retroareolar region, showing no restricted

PATIENT: ASMAA SAYED MAHMOUD YOSEF, 10125268924, Female, 45 y

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٤٠ الحي الأول - الشيخ زايد أمام كلية الهندسة

diffusion. 2nd Look Ultrasound revealed retroareolar prominent ducts.

- Multiple dermal areas of focal thickening with post-contrast enhancement are noted at the the lower quadrants. Second Look ultrasound revealed corresponding dermal circumscribed oval hypoechoic lesions with peripheral vascularity on color Doppler and overlying skin redness, likely inflammatory.
- Normal skin thickness and contour.
- Normal nipple areola complexes.
- Nonspecific axillary lymph nodes.

Opinion:

- **Right axillary tail enhancing pathologically proved malignant mass (BIRADS 6) with ipsilateral indeterminate axillary lymph node.**
- **Bilateral multiple enhancing foci with no restricted diffusion (BIRADS 3).**
- **Bilateral enhancing dermal lesions more pronounced on the left side as described, likely inflammatory for clinical assessment.**

Reported By,

Dr Lobna KHALED.MD

Prof.Dr. RASHA KAMAL

27-Aug-2025 10:43 PM

PATIENT: ASMAA SAYED MAHMOUD YOSEF, 10125268924, Female, 45 y

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4 شارع علوية أول شارع الهرم بجوار داري , الجيزة

مستشفى بهية الشيخ زايد
٤٠ الحي الأول - الشيخ زايد أمام كلية الهندسة

M.R.I

[[DOCTYPE:MRI]]

MRN: 10125268924
Patient Name: ASMAA SAYED MAHMOUD YOSEF
IC No.: 28007250104529
Location: External Clinic
D.O.B: 25/07/1980
Sex: Female

IMMUNOPATHOLOGY

Her2 Receptors

Specimen Id 2508140142
Accession No IMM25-4177
Sample Type Paraffin Block
Collection Date 14/08/2025 10:38 AM
Received Date 14/08/2025 10:39 AM
Doctor Abd ALLAH Mohamed Gamil

Immunohistochemical Report

pathology number
4886-25

Clinical Data
Rt. axillary tail cancer

Gross Picture :
Referred block submitted for HER 2 immunostaining.

System Used :
Ventana BenchMark ULTRA autostainer.

Antibody:
Ventana anti-HER-2 [4B5] rabbit monoclonal antibody.

Scoring system :
Reference: ASCO-CAP HER2 Test Guideline Recommendations, 2018. IHC 0 :as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within $\leq 10\%$ of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within $>10\%$ of the invasive tumor cells. IHC 2+: The revised definition of IHC 2+ (equivocal) is weak to moderate complete membrane staining observed in $>10\%$ of tumor cells.. IHC 3+: based on circumferential membrane staining that is complete, intense in $>10\%$ of tumor cells.

(Laterality) breast Cancer:

Right

Her2 Result

Negative [score 0]



جمهورية مصر العربية
بطاقة تعقب الشخصية

اسماء
سيد محمود يوسف

٢٧ ش راي
حلوان - القاهرة

٢ ٨٠ ٠٧ ٢٥ ٠١ ٠٤٥ ٢٩

KS1723760

national ID - Front